

DCD / WLST COMFORT-MEDICATION NOMOGRAM

[HOSPITAL] SICU · DRAFT v1 · 2026-07-08 — pending approval
Adult opioid/benzo-naïve defaults · one protocol for every withdrawal, donor or not

- 1 PREP** — before family enters
Drips **continue** (never weaned for donation) · **no paralytics** (prior NMB → TOF $\geq 3/4$ + motor recovery) · A-line + 2 IV · **3 h of meds at bedside** · baseline RDOS/CPOT/RASS · time-out; ICU bed held.



PREMEDICATE — T-30 → T-0 min · anticipatory dosing is endorsed; chart it

- 2**
- | | |
|----------------|---|
| Glycopyrrolate | 0.4 mg IV ×1 — secretion control (mild HR rise = side effect, not a target) |
| Morphine | 5 mg IV (range 2–10; ≈ 0.05 –0.1 mg/kg) — anticipatory analgesia / dyspnea |
| Midazolam | 2 mg IV (range 1–5) — anticipatory anxiolysis |
- Heparin** (donation med — separate authorization, care-team RN): **300 U/kg (~30,000 U) IV** at extubation · **hold if actively bleeding.**

GATE ▶ **withdraw only when RDOS < 3** · if ≥ 3 → **treat first (Step 4)**

- 3 WITHDRAW**
Extubate / terminal wean per attending — **dual clearance** (attending + DNAZ coordinator).



TITRATE — THE LOOP · assess & chart q10 min and 10 min after every dose

Trigger — any ONE: RDOS ≥ 3 · CPOT > 2 · RASS $\geq +1$ / restless · HR $\uparrow > 20\%$ **with** a distress sign · new diaphoresis / labored breathing

- 4**
- | | |
|--------------------|--|
| Morphine | 2–5 mg IV q10 min PRN → ineffective at recheck: escalate 2 → 4 → 6 → 8 mg |
| Midazolam | 1–2 mg IV q10 min PRN (agitation) → escalate to 4 mg |
| ≥ 3 boluses/h | start infusion at $\approx 50\%$ of the hour's bolus total (mg/h) — keep bolusing with every rate $\uparrow$ |
| On drips | bolus = 1 h of current infusion + raise rate 25–50% |
| Secretions | glycopyrrolate 0.2 mg IV q2h PRN |
| Refractory | attending → propofol 20–50 mg IV → 10–100 mg/h (or up-titrate existing propofol/dexmedetomidine) |

GOAL ▶ loop until **RDOS < 3** · **CPOT ≤ 2** · **comfortable** · no ceiling · no dose without a charted trigger · never to a target RR/BP/time

DETERMINATION OF DEATH

- 5** Arterial-line **pulse pressure = 0** + apnea + unresponsive ▶ **5-min continuous hands-off** (art line + ECG; restart clock if pulsatility returns; ECG without pulse pressure does not interrupt) ▶ **pronounce** (physician NOT on OPO/recovery/transplant team; TOD = end of observation) ▶ recovery team may enter — never before.



NO DEATH IN THE WINDOW — [120 min / DNAZ organ-specific]

- 6** Stand down → **return to held ICU bed** → **this same protocol continues unchanged** → tissue donation may remain possible. **Anyone may call a pause** for a patient concern, any time.

RDOS — THE TRIGGER (SCORE 8; 0–16; TREAT ≥ 3)

HR (<90 / 90–109 / ≥ 110) · RR (≤ 18 / 19–30 / > 30) · restlessness · paradoxical breathing · accessory-muscle use · end-exp grunting · nasal flaring · fearful face.
0–2 none · **3** mild→treat · **4–6** moderate · **≥ 7** severe→escalate.
Tolerant pt: bolus = 1 h of current infusion (or 10–15% of 24-h opioid). Geriatric: $\sim 50\%$ opioid, same triggers.

THREE RULES THAT KEEP IT DEFENSIBLE

- Never dose to the clock** — donation timing / OR / organ viability are never an indication.
- Never a paralytic** at/after WLST; no OPO/recovery staff ordering or advising comfort care.
- Chart every dose:** score(+components) → drug/dose → comfort indication → 10-min re-score.

ONE PROTOCOL FOR EVERY WITHDRAWAL — DONOR OR NOT. That equal-care fact is the legal shield.

ACCM/Truog 2008 · ATS/ISHLT/SCCM 2013 · VHA 1102.07 · Fast Fact #34 (2025) · Campbell RDOS (Palliat Med 2015) · Dhanani NEJM 2021 · Vacco v. Quill 521 U.S. 793 · A.R.S. §14-1107 · full policy: DRAFT_DCD_policy_comfort_med_protocol.md · not for use until approved.